



THE DEMENTIA HOUSE CALL

A STAGE-BY-STAGE GUIDE FOR FAMILIES

The Early-Stage Guide

What to do after the diagnosis — while there's still time to plan, together

Plain, practical guidance from a doctor who makes dementia house calls

Educational information for caregivers — not medical advice.

HOW TO USE THIS GUIDE

Read only the page you need

This is a broad guide, meant to be read a page at a time. You do not need to read it front to back — start with whatever you're facing now, and come back to the rest when you have room.

THE ONE RULE THAT RUNS THROUGH EVERYTHING

A **sudden** change — new confusion, agitation, or sleepiness over hours or a day or two, often with a fever, pain, a change in urine, or after a new medication — is **medical until proven otherwise**. That is usually delirium, it is frequently reversible, and it needs a phone call, not a behaviour strategy. When something changes fast, think medical first.

A NOTE ON THE FOOTNOTES

A few pages point to products or services that genuinely help at this stage. Those are honest suggestions, not prescriptions — use only what helps, always with your person's knowledge. Some are affiliate links, which means The Dementia House Call may earn a small commission at no extra cost to you; it never changes the guidance, and we never steer a medical decision toward a purchase.

WHERE YOU ARE NOW

You just got the news

A diagnosis of early dementia is a strange kind of grief, because the person is right there in front of you — still themselves, still able, still living their life. And that is the most important thing to hold onto today: **early dementia is not the end of the person, and it is not an emergency.** It is the beginning of a long road on which, right now, you have something you will not always have — **time, and their voice in the decisions.**

The early stage can last a couple of years. Your person can still hold a conversation, follow their routines, and take part in their own life. What's changed is the edges: the missed appointment, the repeated story, the bill that didn't get paid, the trip somewhere new that went wrong. This guide is about using this window well — not to brace for the worst, but to set things up so the harder stages are gentler for both of you.

The single most valuable idea in this guide: the jobs that are easy now become impossible later. Do them now, *together*, while your person can still tell you what they want.

PLAN WHILE THEY CAN TAKE PART

The paperwork that is really about love

This is the work of the early stage, and it is genuinely urgent — not because anything bad is about to happen, but because these decisions need *their* mind while they still have the legal capacity to make them.

THE FOUR DOCUMENTS TO GET IN PLACE

1. **Power of attorney for health** — who speaks for their medical care when they can't.
2. **Power of attorney for finances** — who manages money and bills.
3. **A will**, up to date.
4. **An advance directive / goals-of-care conversation** — what matters to them: what they'd want, and what they'd never want, at the end. (The Care Binder has a gentle, values-first worksheet for exactly this.)

Do it as a conversation, not a transaction. Ask what they want while they can tell you: where they'd like to live if they couldn't manage at home, what "a good day" would need to include, what they're afraid of. Write it down. You are not taking over — you are becoming the keeper of their wishes.

A note on money: early dementia is the season of the financial scam and the forgotten bill. Set up autopay, put a trusted second eye on the accounts, and consider freezing credit. This protects them without stripping their independence.

THE DRIVING CONVERSATION

The hardest early conversation

Driving is independence, identity, and freedom — which is why this is so hard, and why it can't be dodged. Dementia and driving don't mix forever; the question is only *when*, and the goal is to get there safely and with dignity.

- **Watch for the signs:** getting lost on familiar routes, slow reactions, dents and scrapes, confusing the pedals, other drivers honking, needing a co-pilot.
- **Don't do it in one blunt blow.** Frame it as safety for everyone, involve the doctor (let them be the "bad guy" — that's what we're for), and where the law requires reporting, that takes the decision off your shoulders.
- **Replace what driving gave them,** don't just remove it: rides from family, a ride service set up on their phone, a standing lift to the things that matter. Isolation is its own harm.

(The site's driving letter walks through how to have this conversation, US and Canada.)

SAFETY, LIGHTLY, NOW

Small changes that buy years at home

The early stage is the time to make the home quietly safer — before there's a crisis, and while changes can be introduced calmly.

- **Medications:** a weekly pill organiser or a dispenser with an alarm; a plan for who checks it. Missed or doubled doses are one of the commonest early problems.
- **The kitchen:** a stove with an auto-shut-off, or a shut-off device; this is the highest-risk appliance in the house.¹
- **A safety net for being alone:** a medical-alert pendant *while they can still recognise an emergency and press it* — genuinely useful early, and reassuring for you.²
- **Monitoring a space, not a person:** a sensor or camera on the stairs, the front door, or the stove — framed honestly, with their knowledge, as watching a *place* for safety, not surveilling them.³
- **The path to the bathroom** clear and lit; loose rugs gone. Falls prevention starts now.

AFFILIATE NOTE

¹ Home-safety sensors (smoke/CO, leak) — e.g. **X-Sense**. ² Medical alert with fall detection — e.g. **LifeFone**. ³ Space monitoring — e.g. an **Alfred Camera** on the stove or stairs. *Footnotes, not prescriptions: use only what genuinely helps, and always with your person's knowledge.*

TREAT THE HEARING AND THE EYES

The cheapest thing you can do for their brain

This page is short because the message is simple and the evidence is strong: **untreated hearing loss is the single largest modifiable risk factor for worsening dementia**, and poor vision runs a close second. A person who can't hear or see well withdraws, misunderstands, and gets more confused and more agitated — and a great deal of that is fixable.

- **Get their hearing tested and treated.** Modern over-the-counter hearing aids are affordable and genuinely help communication and mood.⁴
- **Get their eyes checked;** update glasses; treat cataracts. Good light everywhere.
- Do it **early**, while they can still learn to use a device — hearing aids are much harder to adopt later.

AFFILIATE NOTE

⁴ Reputable OTC hearing aids — e.g. **Vivtone** (FDA-registered OTC hearing aids). Frame better hearing as connection and risk-reduction, never as a treatment for the dementia itself.

STAY ENGAGED, STAY THEM

Keep the life in the years

The early stage is not a waiting room. Purpose, connection, and movement genuinely slow decline and protect mood — and they make these years worth having.

- **Keep the routines and the roles.** Let them keep doing what they can, even slowly. Capability preserved is capability kept.
- **Move.** A daily walk is one of the best things going for brain, sleep, mood, and balance.
- **Stay social.** Isolation accelerates everything. Old friends, a group, a class, family time.
- **Capture the life story now — this one is urgent.** While they can still tell it, record their history: the names, the places, the way they tell the old stories. A **memory / life-story book** is a gift to everyone who cares for them later (care staff included), and the making of it is a warm project you do *together*.⁵ We ask families to start this early for a reason: it becomes priceless exactly when it can no longer be made.

AFFILIATE NOTE

⁵ Life-story / legacy services — e.g. **Not Forgotten** — or a simple album you build yourselves. The point is the record, not the product.

PROTECTING THE RELATIONSHIP

How you talk matters now

The communication habits you build now will carry you through the harder stages. Start practising them while they're easy.

- **Short, simple sentences; one idea at a time; plenty of pause.**
- **Don't quiz** ("do you remember...?"). It only surfaces the loss. Offer the memory instead.
- **Go with the feeling** when the facts slip. Connection over correction, always.
- **Preserve dignity.** Talk to them, not over them, even as others start addressing you instead.

YOUR OWN OXYGEN MASK, FROM DAY ONE

Start looking after yourself now

The caregivers who last are the ones who build support *before* they need it — not the ones who wait until they're empty. Day one is not too early.

- **Learn the road** (this series is here for that), but don't try to carry all the stages at once. Today has enough in it.
- **Build your bench now:** the family who'll share the load, the friend who listens, a support group, your own doctor.
- **Protect your own mind.** Caregiver stress, anxiety, and low mood are common and treatable — a structured, evidence-based course or a counsellor is a strength move, not a last resort.⁶
- **Name the grief.** You're mourning small losses already. That's normal, and you don't have to do it silently.

AFFILIATE NOTE

⁶ Evidence-based caregiver stress support — e.g. **Stress Control** (Dr Jim White's CBT programme). And the site's free "For you" letter is written for exactly this.

HOW YOU'LL KNOW IT'S MOVING ON

What comes next

The shift from early to middle is gradual, but you'll feel it: when they need help not just with the complicated things (money, planning) but with the **everyday** ones — picking clothes, keeping track of the day, staying safe alone. That's the moment the **Middle-Stage Survival Guide** is built for, with the day-to-day tools for the harder stretch.

For now, though: the person in front of you is still here, still themselves. The most important thing you'll ever do for them, you're doing right now — planning with them, protecting the relationship, and making the ordinary days count.



THE DEMENTIA HOUSE CALL

You don't have to figure this out alone

The Dementia House Call sends free, plain-language letters for families — the same things a good house call would tell you. Join at **thedementiahousecall.com**, and look for the rest of the stage-by-stage series and the Care Binder.

The series: The Early-Stage Guide · The Middle-Stage Survival Guide · The Late-Stage Guide · and the Care Binder, the filing cabinet they all point back to.

This guide is general education for families and is not medical advice; it does not create a doctor–patient relationship and does not replace your own physician. Anything that looks like a sudden change, an infection, or an injury needs assessing, not managing — if you're unsure, assume it does and make the call. Some links in this guide are affiliate links; The Dementia House Call may earn a small commission at no cost to you, and it never changes the guidance. © 2026 The Dementia House Call.